Turnage, Kevin D. (IHS/PHX)

From: Greve, Tonie (IHS/PHX)
Sent: Thursday, June 16, 2022 8:20 AM
To: Turnage, Kevin D. (IHS/PHX)
Cc: Lee, Candace S. (IHS/PHX)
Subject: RE: Notification

Kevin;

I do have reasonable accommodations for my Lung Disease.
Since onboarding and when asked I have forms submitted from the Agency by listing my disabilities. Working here for 11 years, you may have received the information in a email. I do respond and list my disabilities.

2. This information was given to Rebecca and I have asked repeatedly to not have 3 or more broadcast sent at the same time.
As the Direct supervisor you should have been given the copy of the reasonable accommodations for my lung disease and the coverage for the Nurse Assessment/Triage Room. I did speak to you about the E-Comp which I still have not heard back on status from you which as immunocompromised I was placed in the nurse assessment room repeatedly, not a shared duty with all team CCC, contracted Covid.
Broadcast messages sent in sets of 3 or more is a disruption, especially if I am typing a telephone conversation with a patient that I have on the phone and it stops it immediately, there is evidenced base literature and data that can support this as a interruption.

3. I do complete the task but <u>where does it say any email today or in the past that I am the only person to schedule for the Whole Primary Care Medicine Clinic?</u>
Rebecca included all in the email, she initiated it for all team leader to be aware.

4. I have seen where patient information has been broadcasted to all Phoenix Indian Medical Center, and I have had training from this service unit not to do broadcast due to the reasons that if a person logs off it will go out to everyone who is logged on. That is a Hippa violation and a not a Need to Know.
The skype messages, have been addressed when I get it that has patient identification in it, I do inform those who send it not to use it to please put the information in a notification and I will review the information, so to protect the patients identity, this as well if I receive outside emails from patients.
(EBM) has that been approved by Indian Health Service and brought to the attention to HIM Director?

6. I do work effectively and go above an beyond to assist the patients that Indian Health Service Serves. I am held accountable not only for my Official Duties but that of the other team members who shop while at work, watch movies on their phones, turn off their phones, not answer their phones, you have repeatedly sent out emails to staff prohibiting these types of practices, but it still continues.
I do carry out the task of scheduling the appointments not only for the Phoenix Indian Medical Center Case Managers but for patients who repeatedly can not reach anyone else, I have numerous Doctors from within Phoenix Indian Medical Center who contact me as well as other departments as they can not reach the persons who holds official positions but do not respond to the calls or broadcast or skype messages.
Page 2 Paragraph 2 does list my duties but once again it does not say that I am to cover the Whole Primary Care Medicine Clinic, and I did write that all have the same job description listed on the Official Position Description, that is true. I carry out my official duties daily unless on sick leave or vacation, then return to complete the duties that were not picked up by others even when patients repeatedly try to reach the employees who are on their phones. This was in your previous emails.

You Hold me accountable to do my official duties, and (other duties as assigned), your disparate treatment is very evident from this email, you have also have shown and made racist comments to me and disparate treatment is very evident.

I do try to resolve problems at lowest and communicate, only to get this email response or email response in the past. Now you want me to complete a reasonable accommodations for this simple request which I will contact George Patton to complete

I am including the next line in my chain of command as your hostility towards me as well as the racist comments leads me to this so that all are aware.
I am more than willing to sit down with anyone to find resolution that is my professionalism.

Respectfully,

Tonie Greve RN CCC Silver & Ocotillo Teams

From: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>
Sent: Wednesday, June 15, 2022 9:49 PM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Cc: Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>; Patton, George W (IHS/PHX) <George.Patton@ihs.gov>
Subject: Re: Notification
Importance: High

Tonie,

I have removed Mrs. Carrillo and Ms. Tapahe from this email chain since they do not need to be part of this conversation. I have also added George Patton as I am asking him to provide me with the information on any Reasonable Accommodations that the agency has agreed to since I am not aware of any accommodations, and none have been shared with me, so I was unaware of any conditions up until I received this email. There are several issues you have raised in this email, and I am going to attempt to address them systematically.

1. I understand that the changes being made can be frustrating and difficult to adjust to. Please know that the changes are being made for the good of our patients and have been researched and are evidence-based.
2. You state in your email " I do have ADHD; the agency and you are aware as I disclosed this to you." Me and you have never had any type of conversation regarding any of your medical problems or accommodations so I am not sure who this statement is directed to, but I am your supervisor and should be the one you are having these conversations with. With that caveat being stated the Job Accommodation Network lists reasonable accommodations for ADHD to include providing of private workspace (which the agency has provided), additionally recommends providing uninterrupted work time (which can be accomplished but shutting your office door) the broadcast messages are not considered an interruption as they are part of your duties and requirements for accomplishing your duties. Is there anything else we can provide to assist with accommodating you? Recommendations include to-do lists, assistance with prioritization, the use of EAP and structured breaks. I am available to discuss options if you wish.

3. Regarding notifications from inpatient case management, we had a conversation via email about this on May 9, 2022, when I sent out directives on how the inpatient case managers were to notify PCMC employees on the need for appointments for both emergency room and hospital discharges. At which time I informed you regarding this process "This is a new process and may have to be changed but we can discuss as a group to determine if there is a better more efficient way to ensure that CCC's are helping to get follow-up discharge visits scheduled prior to discharge. The broadcast serves as notice of the need of your assistance with scheduling." You stated there was not a reason to have a conversation regarding the process, specifically you stated, "Why would we discuss this in a group when a process has been used in the past?" I informed you that this was a management directive that was expected to be followed. You have not made any mention of this directive again until today when you sent the initial email to Ms. Carrillo and she informed you that she was following the directions given to her by me on how to contact CCC's and the Empanelment nurse (which are still the same as when we discussed this on May 9, 2022 at the time you said that this directive "this seems more of harassment, bullying not assisting patients, and if prevents me from doing my job. I don't get this broadcast action from any other PIMC or PCMC team member." I explained that the current processes were not working, causing patient delays, and must be changed. The process is the same for the other RN's in PCMC and is not any different from what you are being asked to do and is the equitable expectation of your position and all Clinical Care Coordinator positions.
4. You state that if you log off then the broadcast goes to others currently logged in. I have used EHR for 11 years and never seen this occur. Can you please provide more insight so I can have our informaticists investigate this as if occurring it would be a HIPAA issue that will need to be addressed across all IHS if we can recreate the issue. I would like to inquire as to why when I sent out general information on May 12, 2022, again instructing staff to use broadcast or telephone as some employees were putting PHI/PII in skype messages which we have be told is unsecure and not allowed. Additionally, since we must use a separate email to send information encrypted that broadcast is the most efficient and secure manner of having discussions regarding patients and provides an audit trail as to what occurs. Why did you not mention this issue then? Broadcasts stay until closed and can be moved around so as not to interfere with any other work being done.
5. There would not be evidence-based material (EBM) on the use of broadcasting in our EHR since it is proprietary to the government however there is a plethora of EBM on the use of broadcasting to improve communication between teams, patients, and families. I am willing to assist you in finding some of the most current EBM regarding broadcast messaging in the ambulatory setting.
6. You responded to information I sent out on May 6, 2022, regarding BPRM and the scheduling of patients and claimed that you were being required to take on additional duties not in your position description and blaming other individuals that do not have any say or control in how I am running primary care. I responded to your questions as requested and explained my philosophy and my expectations for the CCC's and how AHRQ considers care coordination to be a necessity to properly care for our patients. I explained the philosophy I am trying to instill at PCMC as "Our philosophy is to help our patients achieve the highest level of wellness and independence. By working together with other team members, we can meet our patients' and family's' healthcare needs. If we are saying that is not my job, then we are not focused on the patient." As stated in my email, this may

be temporary, and some duties as assigned are due to staffing issues. At times your email responses have the appearance of being hostile and threatening in tone and demeanor as previously stated when discussing your official position description and I informed you that the position description for Clinical Care Coordinator (PCMC) on page 2 paragraph 2 beginning on line 7 states that the CCC "Assists with obtaining lab results, medical tests, discharge summaries and other information within the hospital and other facilities and assists with arranging appointments, procedures and discharge planning." To wit you did not respond or request clarification. This email was the first mention of the issue again.

7. The response to Ms. Carrillo has the appearance of being unprofessional, rude, and hostile. I have sent these types of emails in the past and have had to learn from them and how to make changes so as not to give that appearance and I can provide you some of the advice and tips given to me to assist you in avoiding making some of the same mistake I made. On May 5, 2022, I asked you that when there were issues "In the future, please send these types of issues to me or just me and leadership" and not to involve others unnecessarily as it creates a hostile work environment and creates more issues than just the subject you are addressing. I again ask not to include entire buildings or departments when there is an issue especially since there are numerous changes occurring rapidly and you may not be aware of the bigger picture. As stated, several times since I took over PCMC on March 28, 2022, I value your input and insight to the way things are occurring, I am asking that in the future you give me an opportunity to explain my rationale and rectify any issues or perceived issues without involving others.

I hope I have answered all your questions and concerns. As always do not hesitate to contact me if there are any further questions or concerns in which I can assist.

Very Resptfully,

Hon. Kevin D. Turnage MSN Ed. RN, AMB-BC, NHDP-BC, NPD-BC

From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Date: Wednesday, June 15, 2022 at 3:09 PM
To: Carrillo, Rebecca J. (IHS/PHX) <Rebecca.Carrillo@ihs.gov>, Tapahe, Carolyn R. (IHS/PHX) <Carolyn.Tapahe@ihs.gov>
Cc: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>, Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>
Subject: RE: Notification

You are sending the Un-assigned team, and the algorithm has Brenda Barr, but you sent me request which I scheduled the appointments. I lost one broadcast appointment today, **which I found when I did the ICARE ER/HOSPITAL F/u** whereas Roxanne did schedule the appointment.
You put Un-assigned patient in a notification and I completed the task. I zone in on new notifications from others who do not work at primary care medicine clinic.

HIPAA compliance, if you or anyone sends a notification and I log off on the Electronic Health record the patient's identification goes to anyone who is logged onto the Electronic Health Record, and it shows the patient's identification chart number and the illness.
All can disagree with my pointing this out but not everyone logged into Electronic health record is a **"Need to Know".** This is under HIPAA compliance under the Indian Health Manuel and under HIPAA rules & regulations.

Sending broadcast after broadcast is not faster getting any appointments but it blocks me from performing my job**, there is no evidenced based literature or data** to support this type of scheduling appointments.

I do have ADHD, the agency and you are aware as I disclosed this to you. Professionally I asked all of the inpatient case managers to send a notification or call me, I do answer my phone and if I am on with a patient , I return calls.
Your discriminating against me, it's a form of harassment (non-sexual) since I asked you not to do this, but you continue to do so.

I also have (2) panels 8 to 10 doctors, providers that I am working with daily on phone or even person I huddle with team members.

Thank you for responding;

Respectively,

Tonie Greve RN CCC Silver & Ocotillo Team

From: Carrillo, Rebecca J. (IHS/PHX) <Rebecca.Carrillo@ihs.gov>
Sent: Wednesday, June 15, 2022 2:38 PM
To: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>; Tapahe, Carolyn R. (IHS/PHX) <Carolyn.Tapahe@ihs.gov>
Cc: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>
Subject: RE: Notification

I'm just following the instructions Kevin Turnage provided us to "Notify CCC via broadcast or phone" according to the standardized algorithm sheet that he sent to our team. If the guidelines have changed please update the standardized algorithm sheet for our group. I apologize, but on our end it can be confusing as changes are made without sufficient communication.
Thank you, Rebecca

From: Greve, Tonie (IHS/PHX) <Tonie.Greve@ihs.gov>
Sent: Wednesday, June 15, 2022 9:40 AM
To: Carrillo, Rebecca J. (IHS/PHX) <Rebecca.Carrillo@ihs.gov>
Cc: Turnage, Kevin D. (IHS/PHX) <Kevin.Turnage@ihs.gov>; Lee, Candace S. (IHS/PHX) <Candace.Lee@ihs.gov>
Subject: Notification

Good Morning Rebecca;

I lost the broadcast information you sent, again I cannot express how easily this happens.

Professionally, I do not work daily by getting broadcast after broadcast, that is not how I was trained nor work daily at Primary Care Medicine Clinic, by doing this repeatedly, you are forcing the way you work onto me without consideration.

Can we find common ground to assist the patients we serve here at Phoenix Indian Medical Center? These actions are more bullying and intimidation.

Can we collaborate and work together? I do have (2) panels and work with (8) doctors daily, as well as get notifications for the Un-assigned team, and the Specialty Clinic.

If the patient is not heading out the door, a notification can be reviewed and an appointment made. I have repeatedly scheduled appointments only to find out the patients were not discharged and the dates of the scheduled appointments do not fall within the 7-10 days.

I review the notifications throughout the day zoning in on the request from persons that do not work at primary care medicine clinic, and it is very distracting when I get broadcast instead of notifications especially when on a phone with a client and typing verbatim what the patient is saying my normal reaction is to delete the broadcast.

I am asking you once again professionally, can you put the request in a small notification, or call me, the broadcast has no information other than needing an appointment.

Respectfully,

Tonie Greve RN CCC Silver & Ocotillo Teams